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Representations and lived experiences of sexual health in women with lung cancer: a qualitative study (SHe-CLu)

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CONTEXT:

Lung cancer is the **3rd most common cancer** in France.[1]

↗ incidence in **women** : **+5% per year** between 2010 and 2018[1]

Medical treatment in cancer → **deterioration in sexual health**[2,3]

American women report :
- **fatigue**
- loss of **stamina**
- shortness of **breath**
- **body changes**
- early **menopause**
- problems with **sexual response** & **desire**
- reduced **orgasm**[4]

AIMS & METHODS:

The aim is to document **how women with lung cancer perceive and experience their sexuality** to better understand their daily lives and refine their support.

1 **focus group** (n=2)
3 questions regarding :
- their **definition of sexuality**
- the **impact of cancer** on their sexuality
- their **experience of solitary sex**



Paula
49 y.o.
Diagnosed: 1 y., 10 m.



Anna
38 y.o.
Diagnosed: 2 y., 10 m.

Thematic analysis[5]

RESULTS:

680 codes in **8** themes and **31** sub-themes have been identified. These following themes were explored in relation to our research question:

Sexuality in the context of cancer
- Definition of sexuality: **consensual** and **shared** pleasure, crucial in general life
- Difficult experience of **loss of sexual desire**
- Different effects based on treatments and their toxicities.
- Fear of sexual function and desire **not returning after the illness**
- Importance of **discussing sexuality**, a taboo subject, in the care journey



"Yes, really, I don't see... My body doesn't have its place, well, there - on the D-day - in a sexual act, you know. [...] No, I won't go down that path, you know, I do everything to avoid going down that path."

"I avoid it, it's true that I avoid the moment of going to bed. Honestly, I avoid it... Whereas before, it happened naturally... But now, well, I really wait until he's asleep to go to bed."



The relationship with the partner
- **Supportive** partner
- "Normal" couple sexuality: a sexuality involving **both partners**
- **Tension** within the couple regarding this matter, primarily from the patients rather than the partners
- Conflicting desires and **avoidance of situations** that may lead to sexual intimacy

Change in body image
- **Weight gain** and **swelling** that retrigger old insecurities
- Port-a-cath (**PAC**): a constant reminder of cancer and its contact evokes disgust
- Premature **aging**
- **Loss** or undermining **of identity**: no longer accepting oneself, unable to look at oneself in the mirror or in the shower

"I see all these [...] mothers of children my age, but I don't actually look like them! Everyone has their own life journey, that's evident... but yeah, I don't look like what I had in mind [...] for myself at that age."



"Because you have an interest in it, but I also saw the benefit of meeting Paula and potentially meeting others at the Hospital."



"Also, to know if there are other people like me!"



Relevance of support groups
- Focus group setting as **a support** to address sexuality
- Importance of **support groups** in providing a safe space to finally discuss sexuality and intimacy

DISCUSSION:

Sexuality of women with lung cancer is undermined. They report **significant changes in their bodies** and **a breakdown in their intimate relationships**. They feel **the need to discuss it**, but there are few places within the hospital setting to address this issue.

Limitations:

The limitations of this study include **the extremely small number of participants** and **their lack of diversity**. This prevents data saturation and only allows for a preliminary response to the issue addressed in our study.

Clinical implications:

Implementing **discussion groups on this subject** could be a solution to address the loneliness experienced by patients.

Research implications:

Continued exploration of this subject among **diverse patient profiles** and **integration into broader research** can provide valuable insights and connections to related issues.

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